

radiological images can at times be misleading. Often, a chapter about a particular topic will feature images of other diagnoses in the differential. For example, the "Ice Pick Headache" chapter features magnetic resonance images of breast metastases to the brain.

The text is organized into sections based on regional anatomy. Readers may have benefited if section prefaces had been included to discuss differential diagnoses of the common causes of pain and to present explanations of the standard workup for pain in each of these anatomical regions. Readers should be aware that no references are cited for the information presented. The text would be a useful resource to primary care physicians caring for a large number of patients with chronic pain. The language is clear and concise, and the subject matter is broad. The section on chest pain syndromes, for example, is thorough in its listing of disorders such as "the devil's grip," sternoclavicular syndrome, sternalis syndrome, and serratus anterior muscle pain syndrome.

However, some sections recommend procedures that demand more expertise. For example, in the chapter on cervicothoracic interspinous bursitis, Waldman discusses administering corticosteroid and bupivacaine via a needle advanced through the supraspinous ligament into the interspinous ligament without advancing the needle into the epidural, subdural, or subarachnoid space. Waldman should caution the reader that this is a superficial trigger point injection. If there is any concern about penetrating the ligamentum flavum, other tools for needle localization, such as fluoroscopy or those supplies found in an epidural kit, are necessary. In addition, for some of the procedures Waldman recommends, such as those for supraspinatus tendinitis vs infraspinatus tendinitis vs subacromial bursitis, live imaging might be beneficial.

The chapters on hand and wrist pain and on foot and ankle pain are robust. In several chapters, such as those on triangular fibrocartilage tear syndrome, scapholunate ligament tear syndrome, and lunotriquetral instability pain syndrome, Waldman writes, "Ultimately, surgery is the treatment of choice." However, the surgical outcomes have been mixed, depending on the extent of instability in each of these injuries.

The book's greatest value is that it discusses syndromes not routinely considered in the differential diagnosis of chronic pain. Because the figures are so vivid, it is possible for practitioners to quickly thumb through the text to review the common presentations of uncommon pain syndromes.

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IMPULSE CONTROL DISORDERS: A CLINICIAN'S GUIDE TO UNDERSTANDING AND TREATING BEHAVIORAL ADDICTIONS

By Jon E. Grant
209 pp, \$26.95
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IMPULSE CONTROL DISORDERS ARE COMMON AND DISABLING behaviors experienced by approximately 5% to 15% of the US population, or between 14 million and 42 million persons. These disorders include pathological gambling, kleptomania, intermittent explosive disorder, pathological hair pulling (trichotillomania), and pyromania. Other disorders such as compulsive Internet use, compulsive sexual behavior, pathological skin picking, and compulsive buying have been proposed as belonging to the same category. Impulsivity is defined as a predisposition toward rapid, unplanned reactions to either internal or external stimuli without regard for negative consequences. Impulse control disorders share common core qualities, including repetitive or compulsive engagement in a behavior despite adverse consequences, diminished control over the problematic behavior, an urge or craving state prior to engagement in the behavior, and a hedonic quality during the performance of the behavior in question. Subjective distress and impaired functioning are often a consequence of impulse control disorders, and their avoidance feeds the cycle of repetitive behaviors. Patients with these disorders continue to struggle with their desire to engage in the behavior and their need to stop because of mounting social, occupational, financial, or legal consequences.

The similarities between impulse control, obsessive-compulsive, and addictive disorders suggest a common neurobiological dysfunction amenable to uniform pharmacological treatment. Biochemical, functional neuroimaging, and genetic studies have implicated multiple neurotransmitter systems in the pathophysiology of impulse control disorders, which may explain why most pharmacotherapy approaches have failed to demonstrate consistent efficacy. The author therefore suggests that further subtyping of those disorders may lead to individualized and targeted therapies.

The preponderance of clinical evidence regarding impulse control disorders suggests the existence of a neurobiological feature also present in diseases of addiction. Therefore, impulse control disorders could be considered behavioral addictions. The positive treatment response obtained with opioid antagonists and glutamatergic agents suggests that the dopamine and (possibly) the glutamate systems affected by these medications may play a role in impulse control disorders as well as in substance abuse and dependence. In the absence of conclusive evidence, prospective studies about the relationship between these disease entities should be conducted to understand the relationship between impulse control and substance abuse disorders. Shame

and secrecy, combined with clinicians' lack of knowledge, complicates the assessment and treatment of these disorders. This increases the likelihood of misdiagnosis and inappropriate treatment. The more common misdiagnoses include mood disorders and obsessive-compulsive disorders. The author points out that persons with impulse control disorders feel pleasure from their behavior, whereas those with obsessive-compulsive disorders generally engage in their behaviors to alleviate anxiety. Therefore, the use of serotonergic medications may relieve the symptoms of obsessive-compulsive behaviors but not symptoms resulting from impulse control disorders.

The use of multiple assessment measures, including clinician and patient-rated symptom severity scales, will contribute to an objective and valid diagnosis of those disorders. Due to the paucity of controlled clinical trials, the treatment of impulse control disorders poses a substantial challenge. The author emphasizes that the systematic study of treatment efficacy and tolerability is in its infancy. Most published studies are based on relatively small sample sizes and are of limited duration, and the heterogeneity of treatment samples may also complicate the identification of effective treatments.

It is likewise important to understand that the adverse consequences of impulse control disorders affect and influence family members. Financial problems as well as physical and psychological abuse are often associated with such disorders and require professional intervention and treatment. Therefore, family therapy should be an integral part of a comprehensive treatment approach.

Legal consequences related to kleptomania, pathological gambling, and pyromania can be especially problematic. Mental disease alone does not exculpate a defendant from criminal responsibility. An insanity defense requires proof that the mental disease bears a relationship to the criminal behavior. Current case law emphasizes that the mental disorder must significantly impair the defendant's capacity to control his or her conduct at the time of the offense. Further revision of criminal statutes may depend on continuous advances in neuroscience, especially if brain imaging studies can demonstrate objective abnormality in brain functioning at the time of the criminal behavior.

This book provides a detailed understanding of impulse control disorders, clinical manifestations, their underlying genetic and neurochemical basis, and options for assessment and treatment. A concluding chapter would have enhanced this excellent book, and the author should consider its inclusion in a second edition.

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POETS ON PROZAC: MENTAL ILLNESS, TREATMENT, AND THE CREATIVE PROCESS

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POETRY IS RISKY. WANT PROOF? PONDER THE PROFILES OF A few renowned poets. Sylvia Plath killed herself at age 30 years, putting her head in the oven with the gas turned on. At age 45, Anne Sexton committed suicide inside her car in the garage—death by carbon monoxide poisoning. John Keats perished from tuberculosis at the tender age of 25. As a group, poets tend to die at age 62. So if you must write, think about a genre safer than poetry. Choose the category of nonfiction, for example, and your statistical lifespan increases to 68 years. Pity the poet. When not succumbing to gas fumes and premature death, a majority still face a serious health threat. One study reports that as many as 80% of poets have a mood disorder.

Poets on Prozac: Mental Illness, Treatment, and the Creative Process looks at the tangled relationship between writing, mental illness, and psychiatric intervention. The anthology consists of "firsthand accounts written by poets as they describe the impact of psychiatric treatment on their creative process." Two questions dominate the discussion: Does mental illness help or hinder creativity? How do psychotropic drugs affect creative writing? Sixteen current poets reveal their experience with psychotherapy and prescription medications and explain how psychiatric treatment has influenced their writing and altered their creativity.

Each essay is a sort of mini-memoir that incorporates samples of the author's poetry. The list of psychiatric problems afflicting these poets looks a lot like the index of the *Diagnostic and Statistical Manual of Mental Disorders* (Fourth Edition, Text Revision): bipolar disorder, postpartum psychosis, anxiety, obsessive-compulsive disorder, major depression, schizophrenia, eating disorders, alcoholism, and posttraumatic stress disorder. Most of these essays are mildly interesting. A few have catchy titles, such as "Chemical Zen" and "How I Learned to Count to Four and Live with the Ghosts of Animals." The most beguiling of the bunch, "Perfecting the Art of Falling," opens with a poem and a jolting recollection: "I had jumped out of a four-story hospital window, in 1966, in Aix-en-Provence, France." The essay proceeds to chronicle the turbulent life of a man with schizophrenia, including his half-body cast, a jumbo dose of chlorpromazine, and 3 significant R's: rage, relocation, and redemption. Many of the contributors to this collection identify the act of writing as a form of therapy. Some of the poets readily admit to their apprehension of taking psychiatric medication. The bulk of these essays are imbued with sadness, frustration, resentment, or anger. Some of the authors identify their emotions and